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Medicine

Medicine Letter 1

Read the case notes and complete the writing task which follows

John Elvin is a 48-year-old patient in your General Practice

5/05/11

Subjective: Complaint of occasional mild central chest pain on exertion
Has mild asthma but otherwise previously well
Nil family history of cardiac disease
1 pack day smoker and drinks 10 standard drinks 5/7
Under significant stress with own business
Medications – seretide two puffs BD salbutamol two puffs prn
Allergies - Nil

Objective: Nil chest pain O/E
ECG NAD
Troponin level NAD

Assessment: Early stages of IHD
D/D - stress related chest pain
Alcohol dependence but not interested in changing

Plan: Check serum lipids
Refer for exercise stress test
Review in 1 week

12/5/11

Subjective: Still only very occasional chest pain on exertion
Has runny nose & pharyngitis at present with ↑ asthma symptoms
Attended stress test with very mild chest pain at high exercise load

Objective: Some very slight ischaemic changes present in exercise test
Mild bilateral wheeze present
Cholesterol mildly ↑

Assessment: Ischaemic heart disease/angina
Viral upper respiratory tract infection

Plan: Commence on lipitor, nitrates(imdur), aspirin and prn anginine
Educate anginine use
Review in 2/52

26/5/11

Subjective: Chest pain for the last week
Still c/o frequent mild wheeze
Often forgets to take seretide puffers because of ETOH consumption

Objective Mild bilateral wheeze still present

Assessment Mild Asthma 2° to ↓ compliance with medication
Alcohol dependence now affecting medication compliance

Plan Emphasised importance of preventative anti-asthma meds
Recommended pt write put a reminder for asthma and all medications on his fridge.
Encouraged pt to use prn salbutamol until asthma improves
Offered ETOH dependence treatment pharmacotherapy- will consider this.

1/6/11

Subjective: Passing by medical centre and c/o sudden onset crushing chest pain on background of URTI and worsening asthma since last
Not relieved by anginine
Very audible wheeze

Examination ECG – mild ST elevation in anterior leads. ST 120
Lungs – O/A moderate wheeze and mild bilateral crackles. SP O2 86% on R/A
Heart – Slight S3 sound +ve

Assessment Likely anterior AMI; ? triggered by respiratory issues
Acute exacerbation of asthma 2° to URTI
? Mild APO

Plan Paramedic transfer to ED
O2 15L via non-rebreather (pt isn't CO2 retainer)
GTN patch applied
IV morphine 5mg given
Ipratropium Bromide 500ug given via nebuliser in view of tachycardia
Frusemide 40mg given

Notes

Writing task

Using information provided in the case notes, write a referral letter to Dr Jeremy Barnett, the Emergency Registrar on duty at Maroubra Hospital, Lakes Rd, Maroubra.

In your answer:

- Expand the relevant notes into complete sentences
- Do not use note form
- Use letter format

The body of the letter should be approximately 180-200 words.

Medicine Letter 2

Read the case notes and complete the writing task which follows

Yuxiang Meng is a 21 year old overseas student chef from China in your general practice. He only speaks very basic English and sees you because you are a GP from a Chinese background and speak Mandarin.

2.03.11 Chief complaint - URTI symptoms for 5 days.

O/E: *Mild pharyngitis & rhinorrhea. T 37.5

*C/O chronic insomnia

*Observed to be elevated in mood, tangential & ? delusional about fixing the world's nuclear waste problem

*Nil obvious signs of organic syndromes

Assessment: Mild viral illness & ? mania/1st episode BPAD

Plan: Nil treatment for URTI, just rest & ↑fluid intake. Referral made to local community mental health for urgent assessment. Pt. escorted home by his uncle. Diazepam 10mg QID prescribed & to be given with community MH team's supervision.

Investigations (exclude organic pathology & baseline)

-FBC

-UEC

-TFTs

-LFTs

-CMP

-urgent CT scan

3.03.11 Mental health team used interpreter and concur with provisional diagnosis of mania. They state the following: no immediate dangers to self/others; MH keen for GP involvement due to language issues and they will monitor pt. daily; they are keen to avoid hospitalisation as pt. very afraid of idea of psych. ward due to stigma of the same in China
Today pt's uncle accompanied pt. to GP surgery get blood results.

O/E * Bloods NAD except mildly ↓protein & mild hypokalaemia (3.2 K+)

*CT NAD

*MSE – still tangential and delusional about same theme, but only mildly elevated since sleeping well post diazepam

Assessment: Likely non-organic mania

Plan:

- *Commence pt. on quetiapine 50mg BD (starting dose)
- *↓diazepam to 10mg either BD or TDS depending on MH team's assessment.
- *R/V in 3/7; likely ↑of quetiapine.
- *Commence pt on K+ (Span K) tablets.

7.03.11 Pt. was relatively settled for 3/7 but uncle suspects he has secreted & discarded meds. Last night stayed up all night singing Chinese revolutionary songs (not usual behaviour) and running naked down his street. Uncle didn't want to call MH for fear of 'getting locked up'.

O/E

- * Pt very elevated in mood, pressured in speech, loose in associations and fixated on having to rid Australia of all nuclear waste by tomorrow. Believes he can draw power from Mao Ze Dong's spirit to achieve this.
- *Pt stripped naked in front of GP and tried to hug him.

Assessment Acute manic episode

Plan:

Offered stat quetiapine 100 mg & diazepam 20mg but refused.
Schedule pt under MHA
Have uncle accompany pt with ambulance & police to RNSH ED
Refer to on call psych reg Dr Ben Hinds
Update local MH team.
Long term – try to refer to Chinese speaking psychiatrist.

Writing task

Using information provided in the case notes, write a referral letter to Dr Ben Hinds, the Psychiatry Registrar on duty at Maroubra Hospital, Lakes Rd, Maroubra.

In your answer:

- **Expand the relevant notes into complete sentences**
- **Do not use note form**
- **Use letter format**

The body of the letter should be approximately 180-200 words.

Medicine Letter 3

WRITING SUB-TEST DOCTORS

Time allowed: 5 minutes reading time (no writing), 40 minutes writing time

Mrs Daniela Starkovic is a patient in your general practice. Read the case notes below and complete the writing task that follows.

CASE NOTES

Mrs. Daniela STARKOVIC
45 years old, married 2 children

Past history

Migraines
Medications - nil

20/01/07

Subjective

presents with abdominal pain
doesn't like fatty foods
otherwise well

10 days ago

- epigastric pain radiating to R side 1 hour after dinner
- associated nausea, no vomiting / regurgitation
- pain constant for 1 hour
- no medications
- no change bowel habits, no fever, no dysuria

Last night

- recurrence similar pain, worse
- duration 2 hours
- vomited X 1, no haematemesis
- pain constant, colicky features
- aspirin X 2 taken, no relief

Objective:

overweight
T 37° P 80 reg, BP 130/70

mild tenderness R upper quadrant abdomen
no masses, no guarding, no rebound, bowel sounds normal
Murphy's sign neg
Urine – trace bilirubin

Assessment:

?? biliary colic
?? peptic ulcer

Plan:

Liver Function Tests (LFTs)
Biliary ultrasound (US)
R/V 3/7

23/01/07

Subjective:

No further episodes
Patient anxious re possibility cancer

Objective:

LFTs – bilirubin 12 (normal range 6-30)
Alkaline phosphatase (ALP) 120 (normal < 115)
Aspartate transaminase (AST) 20 (normal 12-35)

Assessment: ? mild obstruction

US – small contracted gallbladder, multiple gallstones
Common bile duct diameter normal
Normal liver parenchyma

Assessment: cholelithiasis

Plan: Reassurance re cancer
Referral Dr. Andrew McDonald (general surgeon) assessment, further management, possible cholecystectomy

WRITING TASK

Using the information in the case notes, write a letter of referral to Dr Andrew McDonald a general surgeon at North Melbourne Private Hospital 86 Elm Road North Melbourne 3051. The main part of the letter should be approximately 180-200 words long.

DO NOT use note form in the letter; expand the case notes where relevant into full sentences.

Medicine Letter 4

DOCTORS

Time allowed: 5 minutes reading time (no writing), 40 minutes writing time

Mr Jack Wojovski is a patient in your general practice. Read the case notes below and complete the writing task that follows.

CASE NOTES

Mr Jack Wojovski
43 year old man.

Social History

Job: factory worker 18 years
Home: married
Activities: alcohol: 1 – 2 glasses beer / night
smoking: no

28/12/06

Subjective

Lifting heavy object at work, painful spasm lower back
Reported to factory nurse
Pain persists
No neurological symptoms

Objective

Tender L4 L5 in paralumbar area
Range of Movement (ROM) limited
Straight Leg Raising (SLR) 45° Lower limb reflexes normal
Power, sensation normal

Assessment:

lower back strain

Plan:

rest 2 days, analgesia, heat, Work Cover certificate

02/01/07

Subjective

Pain worse, persistent
Unable to drive or bend
Taking Panadeine 4 hourly

Objective

No change

Assessment:

severe lower back strain

Plan:

Naprosyn, physio

12/01/07

Subjective

Pain relieved
4 physio sessions
Naprosyn 500mg b.d.

Objective

Pain on forward flexion
Full lumbar spine movements. Tender L4 L5
L=R=90°
Power, sensation, reflexes of lower limbs normal

Assessment:

recovering from severe lower back strain

Plan:

Return to work light duties, reduce Naprosyn prn. Continue Physio

17/01/07

Subjective

Pain exacerbated by return to work
Stress in marriage

Objective

Tender L4 L5
Reduced front flexion and extension, SLR 45° L=R, no neurological symptoms

Assessment:

exacerbation lower back

Plan:

X-ray lumbar spine, liaise with physiotherapist, discuss marital problems

20/01/07

Subjective

No change, unable to perform light duties
Physio temporary relief
Wife feels husband over-reacting

Assessment:

Work-related back injury not responding to treatment as expected
Difficult to return to work

Plan:

Refer to rehabilitation specialist

WRITING TASK

Using the information in the case notes, write a letter of referral to Dr Helen Wu at South Melbourne Rehabilitation Services 123 Emerald St, South Melbourne 3205. The main part of the letter should be approximately 180-200 words long.

DO NOT use note form in the letter; expand the case notes where relevant into full sentences.

Medicine Letter 5

WRITING SUB-TEST DOCTORS

Time allowed: 5 minutes reading time (no writing), 40 minutes writing time

Mr Zu is a patient in your general practice. Read the case notes below and complete the writing task that follows.

CASE NOTES

03/01/07

Mr Jing ZU
72 yo man.

Past history

Hypertension 18 years
Ischaemic heart disease 10 yrs
Acute Myocardial Infarction 1999
Congestive Cardiac Failure (CCF) 5 yrs

Family history unremarkable

Medications

Lasix 40mg mane, Enalapril 10mg mane, Slow K TT bd, Nifedipine 10mg tds,
Anginine T sl prn

Social History

Job: retired school teacher
Home: married
Activities: gardening
smoking: no

Subjective

Angina on exertion – gardening, relief with rest and Anginine
Sleeps two pillows, no orthopnoea
Mild postural dizziness

Thin, looks well.
Pulse 84 reg, BP 160/90 lying, 145/80 standing
Jugular Venous Pressure (JVP) + 3 cm
Apex beat not displaced
S1 and S2 no extra sounds nor murmurs
Chest - Bilateral basal crepitations
Abdomen – normal
Ankles mild oedema, pulses present

Assessment: Stable CCF, angina

Plan: Watchful monitoring

15/01/07

Subjective:

↑ dyspnoea, orthopnoea (sleeps on 4 pillows)
↑ ankle oedema
no chest pain

Objective:

BP 140/90
JVP + 6 cm
Chest crepitations to mid zones
Heart S1 and S2
Ankles oedema to knees

Assessment: Deteriorating CCF ? cause

Plan: ECG, ↑ Lasix 80 mg mane, R/V 2 days

19/01/07

Subjective:

Dyspnoea “feels a bit better”
Angina 10 min episode on mild exertion yesterday

Objective:

JVP + 4 cm
Chest fewer crepitations to mid zones
ECG - ? ischaemic changes anterolaterally

Assessment: ischaemic heart disease

Plan: Referral Dr. George Isaacson, cardiologist, management of ischaemic heart

WRITING TASK

Using the information in the case notes, write a letter of referral to Dr Isaacson, a cardiologist at 45 Inkerman Street Caulfield 3162. The main part of the letter should be approximately 180-200 words long.

DO NOT use note form in the letter; expand the case notes where relevant into full sentences.

Medicine Letter 6

OET Practice Writing Test

Read the case notes below and complete the task that follows.

WRITING SUB-TEST DOCTORS

Time allowed: 5 minutes reading time (no writing), 40 minutes writing time

Mr Zu is a patient in your general practice. Read the case notes below and complete the writing task that follows.

CASE NOTES

03/01/07

Mr Jing ZU
72 yo man.

Past history

Hypertension 18 years
Ischaemic heart disease 10 yrs
Acute Myocardial Infarction 1999
Congestive Cardiac Failure (CCF) 5 yrs

Family history unremarkable

Medications

Lasix 40mg mane, Enalapril 10mg mane, Slow K TT bd, Nifedipine 10mg tds,
Anginine T sl prn

Social History

Job: retired school teacher
Home: married
Activities: gardening
smoking: no

Subjective

Angina on exertion – gardening, relief with rest and Anginine
Sleeps two pillows, no orthopnoea
Mild postural dizziness

OCCUPATIONAL ENGLISH TEST PREPARATION ONLINE

Objective:

Thin, looks well.
Pulse 84 reg, BP 160/90 lying, 145/80 standing
Jugular Venous Pressure (JVP) + 3 cm
Apex beat not displaced
S1 and S2 no extra sounds nor murmurs
Chest - Bilateral basal crepitations
Abdomen – normal
Ankles mild oedema, pulses present

Assessment: Stable CCF, angina

Plan: Watchful monitoring

15/01/07

Subjective:

↑ dyspnoea, orthopnoea (sleeps on 4 pillows)
↑ ankle oedema
no chest pain

Objective:

BP 140/90
JVP + 6 cm
Chest crepitations to mid zones
Heart S1 and S2
Ankles oedema to knees

Assessment: Deteriorating CCF ? cause

Plan: ECG, ↑ Lasix 80 mg mane, R/V 2 days

19/01/07

Subjective:

Dyspnoea “feels a bit better”
Angina 10 min episode on mild exertion yesterday

Objective:

JVP + 4 cm
Chest fewer crepitations to mid zones
ECG - ? ischaemic changes anterolaterally

Assessment: ischaemic heart disease

OCCUPATIONAL ENGLISH TEST PREPARATION ONLINE

Plan: Referral Dr. George Isaacson, cardiologist, management of ischaemic heart disease

WRITING TASK

Using the information in the case notes, write a letter of referral to Dr Isaacson, a cardiologist at 45 Inkerman Street Caulfield 3162. The main part of the letter should be approximately 180-200 words long.

DO NOT use note form in the letter; expand the case notes where relevant into full sentences.

Medicine Letter 7

OET Practice Writing Test

Read the case notes below and complete the task that follows.

WRITING SUB-TEST DOCTORS

Time allowed: 5 minutes reading time (no writing), 40 minutes writing time

Ms Janet Bird is a patient in your general practice. Read the case notes below and complete the writing task that follows.

CASE NOTES

Ms Janet BIRD
16 yo girl

Past history

Unremarkable, no medications

Social History

Attends local secondary school, Year 11, lives parents, younger brother

11/11/07

Subjective

Presented alone
Constipation 3 months, 1 X firm bowel action every 4-5 days
Diet includes 2 tablespoons bran in morning, has tried laxatives
Otherwise well

Objective:

Ht. 172 cms Wt. 52 kgs.
Pulse 73 reg, BP 100/50
Abdomen lax, no masses

Pt. Requested prescription for "strongest" laxative. Request refused. Advice re ↑
vegetables, fibre and fluids.

28/12/07

Subjective:

Presents with mother. Mother concerned re Janet's lack of appetite and weight loss.
Family arguments about the situation.

Objective:

Pale, thin. Wt. 47 kgs.
BP 100/60 lying and standing
Abdomen and urinalysis both unremarkable

Plan: Review Janet alone, Tests Full Blood Exam, Thyroid Function, Liver Function

05/01/08

Subjective:

Janet complains parents are "over-reacting". Feels her ideal weight is 40 kgs.
Denies vomiting

Test results: normal

Assessment: Anorexia nervosa

Plan: Referral Dr. Suzanne O'Brien, psychiatrist

WRITING TASK

Using the information in the case notes, write a letter of referral to Dr O'Brien, a psychiatrist at 67 Sigmund Street Brighton 3186. The main part of the letter should be approximately 180-200 words long.

DO NOT use note form in the letter; expand the case notes where relevant into full sentences.

Medicine Letter 8

OET Practice Writing Test

Read the case notes below and complete the task that follows.

WRITING SUB-TEST DOCTORS

Time allowed: 5 minutes reading time (no writing), 40 minutes writing time

Ms Ann Howard is a patient in your general practice. Read the case notes below and complete the writing task that follows.

CASE NOTES

Mrs. Ann HOWARD

36 years old, married 3 children

Past history

Ovarian cystectomy and appendicectomy

Early October 2006 last menstrual period

18/12/06 – left lower abdominal pain

09/01/07 – vaginal bleeding, abdominal cramps. Presented hospital emergency dept
? spontaneous abortion

20/01/07

Subjective

Reported yesterday sudden onset L lower abdo pain, relieved by Valium

Today pain persists, sharp and constant, worse sitting up, walking or bending

No vomiting or nausea, no urinary or bowel symptoms, no weight loss, no change of bowel habits

Objective:

Not distressed

Pulse 96 reg, BP 140/80

Very tender on light palpation L lower quadrant abdomen

Vague mass palpable

Arranged tests: pregnancy test, Full Blood Exam, ESR

21/01/07

Pain persists but less

No bowel motion for 3 days when passed hard stool coated with bright red blood

22/01/07

Subjective:

Pain worse after eating

Objective:

Moderately distressed, abdomen tense
Haemoglobin 9.3 g/dl. Mild left shift
Quiet bowel sounds
No bowel action or flatus

Assessment: Early bowel obstruction ? diverticulitis ? carcinoma

Plan: Referral Dr. Jose Jiminez surgeon

WRITING TASK

Using the information in the case notes, write a letter of referral to Dr Jiminez, a surgeon at Melbourne Private Hospital 19 Grange Road Melbourne 3000. The main part of the letter should be approximately 180-200 words long.

DO NOT use note form in the letter; expand the case notes where relevant into full sentences.

Medicine Letter 9

Read the case notes below and complete the task that follows.

WRITING SUB-TEST DOCTORS

Time allowed: 5 minutes reading time (no writing), 40 minutes writing time

Mrs. Larissa Zaneeta is a patient in your general practice. Read the case notes below and complete the writing task that follows.

CASE NOTES

Mrs. Larissa Zaneeta

38 year old marketing manager, married, one child (four-year-old boy).

Past history unremarkable. No medications

11/07/05

Complains of tiredness, difficulty sleeping for 2 months due to work stress
Plans another child in 12 months, currently on oral contraceptive pill (OCP)

O/E: Appears pale, tired and slightly restless
 BP 140/80
 No abnormal findings

Assessment: Stress-related anxiety

Plan: advised relaxation techniques, reduce working hours, prescribe sleeping tablets tds

15/08/06

Stopped OCP 4 months earlier, still menstruating
Worried
Sleep still difficult, work stress unchanged, not possible to reduce hours

O/E: Tired-looking, slightly teary

Assessment: Work stress, growing anxiety failure to conceive

Plan: discussed nature of conception – takes time, patience
 discussed frequency sexual intercourse
 discussed methods – temperature / cycle

18/01/07

expressed anxiety re failure to conceive, says she's "too old"
sleep still a problem

O/E: crying, pale, fidgety
Vital signs / general exam NAD
Pelvic exam, pap smear

Assessment: as per previous consultation

Plan: 1-2 Valium b.d.
Suggested she re-present next week accompanied by wife.

25/01/07

Mr. Zaneeta very supportive of having another child
No erectile dysfunction, libido normal
Mrs. Zaneeta unchanged

O/E: Mr. Zaneeta normal

Plan: Check Mr. Zaneeta's sperm count

02/02/07

Sperm count normal

Plan: Refer for specialist advice

WRITING TASK

Using the information in the case notes, write a letter of referral to Dr Elvira Sterinberg, a gynaecologist at 123 Church St Richmond 3121. The main part of the letter should be approximately 180-200 words long.

DO NOT use note form in the letter; expand the case notes where relevant into full sentences.

Patient: Anne Hall (Ms)

DOB: 19.9.1965

Height: 163cm Weight: 75kg BMI: 28.2 (18/6/10)

Social History: Teacher (Secondary – History, English)

Divorced, 2 children at home (born 1994, 1996)

Non-smoker (since children born)

Social drinker – mainly spirits

Substance Intake: Nil

Allergies: Codeine; dust mites; sulphur dioxide

FHx: Mother – hypertension; asthmatic; Father – peptic ulcer

Maternal grandmother – died heart attack, aged 80

Maternal grandfather – died asthma attack

Paternal grandmother – unknown

PMHx: Paternal grandfather – died ‘old age’ 94

Childhood asthma; chickenpox; measles

1975 tonsillectomy

1982 hepatitis A (whole family infected)

1984 sebaceous cyst removed

1987 whiplash injury

1998 depression (separation from husband); SSRI – fluoxetine 11/12

2000 overweight – sought weight reduction

2002 URTI

2004 dyspepsia

2006 dermatitis; Rx oral & topical corticosteroids

18/6/10 PC: dysphagia (solids), onset 2/52 ago post viral(?) URTI

URTIs self-medicated with OTC Chinese herbal product – contents unknown

No relapse/remittent course

No sensation of lump

No obvious anxiety

Concomitant epigastric pain radiating to back, level T12

Weight loss: 1-2kg

Recent increase in coffee consumption

Takes aspirin occasionally (2-3 times/month); no other NSAIDs

Provisional diagnosis: gastro-oesophageal reflux +/- stricture

Plan: Refer gastroenterologist for opinion and endoscopy if required

Writing task

Using the information in the case notes, write a letter of referral for further investigation and definitive diagnosis to the gastroenterologist, Dr Jason Roberts, at Newtown Hospital, 111 High Street, Newtown.

Medicine Letter 11

OCCUPATIONAL ENGLISH TEST

WRITING SUB-TEST: MEDICINE

TIME ALLOWED: READING TIME: 5 MINUTES
WRITING TIME: 40 MINUTES

Read the case notes below and complete the writing task which follows.

Notes:

Patient: Mrs Priya Sharma
DOB: 08.05.53 (Age 60)
Residence: 71 Seaside Street, Newtown

Social Background: Married 40 years – 3 adult children, 5 grandchildren (overseas). Retired (clerical worker).

Family History: Many relatives with type 2 diabetes (NIDDM)
Nil else significant

Medical History: 1994 – NIDDM
Nil significant, no operations
Allergic to penicillin
Menopause 12 yrs
Never smoked, nil alcohol
No formal exercise

Current Drugs: Metformin 500mg 2 nocte
Glipizide 5mg 2 mane
No other prescribed, OTC, or recreational

29/12/13

Discussion: Concerned that her glucose levels are not well enough controlled – checks levels often (worried?)
Attends health centre – feels not taking her concerns seriously
Recent blood sugar levels (BSL) 6-18
Checks BP at home
Last eye check October 2012 – OK
Wt steady, BMI 24
App good, good diet
Bowels normal, micturition normal

O/E: Full physical exam: NAD
BP 155/100
No peripheral neuropathy; pelvic exam not performed
Pathology requested: FBE, U&Es, creatinine, LFTs, full lipid profile, HbA1c
Medication added: candesartan (Atacand) tab 4mg 1 mane
Review 2 weeks

TURN OVER

05/01/14	Pathology report received: FBE, U&Es, creatinine, LFTs in normal range GFR > 60ml/min HbA1c 10% (very poor control) Lipids: Chol 6.2 (high), Trig 2.4, LDLC 3.7
12/01/14	Review of pathology results with Pt Changes in medication recommended Metformin regime changed from 2 nocte to 1 b.d. Atorvastatin (Lipitor) 20mg 1 mane added Glipizide 5mg 2 mane Review 2 weeks
30/01/14	Home BP in range Sugars improved Pathology requested: fasting lipids, full profile
06/02/14	Pathology report received: Chol 3.2, Trig 1.7, LDLC 1.1
10/02/14	Pathology report reviewed with Mrs Sharma Fasting sugar usually in 16+ (high) range Other blood sugars 7-8 Refer to specialist at Diabetes Unit for further management of sugar levels

Writing Task:

Using the information in the case notes, write a letter of referral to Dr Smith, an endocrinologist at City Hospital, for further management of Mrs Sharma's sugar levels. Address the letter to Dr Lisa Smith, Endocrinologist, City Hospital, Newtown.

In your answer:

- **Expand the relevant notes into complete sentences**
- **Do not use note form**
- **Use letter format**

The body of the letter should be approximately 180–200 words.

PHARMACY

Pharmacy Letter 1

OCCUPATIONAL ENGLISH TEST

WRITING SUB-TEST: PHARMACY

TIME ALLOWED: READING TIME: 5 MINUTES
WRITING TIME: 40 MINUTES

Read the case notes below and complete the writing task which follows.

Notes:

Personal Details:

Name: Alexia Rollinson (Ms)
Address: 15 Fine St, Newtown
DOB: 12/11/1973
Age: 40 years
Date: 10 February 2014

Social/Family Background:

Single. Works full time as an accountant

Diagnosis: Hypertension, hypercholesterolaemia, low vitamin D since 2011

Medication: Betaloc (metoprolol), 100mg b.d.
Lipitor (atorvastatin), 20mg mane
Ostevit-D 1000IU mane

Current Status: BP 147/100mmHg (taken in pharmacy)
Lipid profile: LDL – 131, HDL – 64, Triglycerides – 269mg/dl
Vitamin D < 54 (60-160nmol/L) (print out with customer)
Ht 153cm, Wt 65kg, BMI 27.8 (verbal from customer)
Does no regular exercise – drives to work, no sport or recreational activity
Low mood
Overweight

Discussions in Pharmacy:

New to area, moved 1 month ago, and has no GP yet.
Medications required today and repeats are filled.
Came in for advice and explained current needs.
Monitoring diet to decrease Wt – target 58kg, BMI <25.

- Exercise – Started own exercise program (e.g., walk 30 min 4 times/wk). Says 'never sticks to it'. Has tried all types of exercise aids advertised on TV, video programs, getting desperate & upset. Wants some help due to lack of progress.

- Diet – Discussed fruit & vegetables, low fat milk, low GI foods & low saturated fats. Bought two electronic scales last week, one for kitchen (food) & one for bathroom (self). Discussed fruit & nut snacks, not chocolate bars (admitted to loving them). Always browsing for Wt loss products. Tried several tablets, drinks, powders, etc. Getting desperate & upset. Wants help due to no progress with Wt loss or change in exercise & daily activities.
Offered to write to local GP for support. Also mentioned a dietitian – customer liked idea.

Pharmacy Management:

- Provided free booklets
 - Healthy eating and exercise
 - Council brochure on walking tracks, walking groups, etc.
 - Local gymnasiums & sports groups
- Letter to GP – suggested referral to dietitian

Writing Task:

Using the information in the case notes, write a letter of referral to Dr Sally Windwood, 9 Blewston St, Newtown, to explain your discussion and advice including a suggestion of consulting a dietitian.

In your answer:

- **Expand the relevant notes into complete sentences**
- **Do not use note form**
- **Use letter format**

The body of the letter should be approximately 180–200 words.

Pharmacy Letter 2

OCCUPATIONAL ENGLISH TEST

WRITING SUB-TEST: PHARMACY

TIME ALLOWED: READING TIME: 5 MINUTES

WRITING TIME: 40 MINUTES

Read the case notes and complete the writing task which follows.

Notes:

You are a pharmacist at Newtown Hospital. An elderly patient who has been treated for a fractured femur is being discharged. You are writing a letter to her carer (her daughter) to ensure the medication regime is followed when she returns home.

Patient History

Name: Mrs Alice Ramsey

Date of Birth: 4 January 1925

Allergies: Nil

Current Medication:

On Admission: Zantac (ranitidine) (for GORD): 150mg bd

Lipitor (atorvastatin): 20mg mane (on empty stomach)

On Discharge: Zantac (ranitidine) (for GORD): 150mg bd

Lipitor (atorvastatin): 20mg mane

Heparin low molecular weight (LMWH) (anti-coagulant): 7500 bd – to be continued until mobile

Panadeine Forte (paracetamol & codeine for pain relief): 500mg 4-hourly/prn

Durolax (to prevent constipation): 10mg nocte

Maxolon (metoclopramide) (for side effects of codeine): 10mg tds/prn

Penicillin (prophylactic: ↓ risk of post-op infection): 250mg qid 2/52

Drug Information: *Adverse Drug Reactions*

Ranitidine *Adverse:* headache; GI upset; rash; CNS disturbances (rare)

Atorvastatin *Adverse:* Serious: rhabdomyolysis, myopathy, myalgia (0.2%);
GI upset (1%); headache (2%); rash (2.5%);
flu-like symptoms (1.5%); raised LFTs (1.3%)

Heparin *Adverse:* haemorrhage, easy bruising, nausea, vomiting

Codeine/Paracetamol *Adverse:* constipation, stomach-aches, nausea, vomiting;
Rare: dependence, tolerance; CNS disturbances incl. impaired alertness

Metoclopramide *Adverse:* CNS disturbances incl. impaired alertness (rare); tardive dyskinesia (rare)

Penicillin Adverse: Rare: sensitivity reactions; haematological effects; nausea, vomiting, mild diarrhoea; allergic skin rash or hives

Social History:

Pt normally lives alone. On discharge, staying with daughter.
Pt non-driver. Public transport.

Relevant History for Surgical Procedure:

Height 168cm; Weight 75kg; BMI 26.8

Non-smoker

Dentures – upper & lower

Gastro-oesophageal reflux disease (GORD) – controlled by medication

Hypercholesterolaemia – controlled by medication

11 July 2010

2:45pm: Pt brought to A&E by ambulance. Knocked down by car in Garden Nursery car park (buying plants) – landed on bitumen. Driver failed to see her in rear-view mirror → reversed into her. Not run over. Fell on R side on femur.

Presenting symptoms: pain – post fall & difficulty standing or walking

3.00pm: Pt seen by Dr Hogarth. Pain relief: pethidine (opiate)

X-rays of affected femur – anterior-posterior & lateral views

Repeat films with hip at 15-20° internal rotation → MRI

5:30pm: Transferred to ward

Pt booked for surgery 12 July am – nil by mouth from midnight

Full pre-operative general investigation: LFTs, platelet count, WBC count, WBC types, RBC count, RBC indices, Hg, haematocrit, blood smear, ECG & chest X-ray

12 July 2010

Open reduction & internal fixation (ORIF) performed

GA given: induction – propofol; sevoflurane, fentanyl, midazolam, suxamethonium, ondansetron

Heparin – thrombus prevention

IV antibiotics – penicillin – continued for 24/24 post surgery

Immobilised with spica cast

Post-Op

TURN OVER

- pressure sore prevention & care of pressure areas; wound care
- pain relief
- fluid balance & blood loss monitoring: IV fluids + penicillin
- nutritional management: oral protein supplementation
- thrombus prevention: low dose, low molecular weight heparin, & mechanised compression stockings
- lower limb circulation & sensation
- early mobilisation & weight bearing on injured leg

24 July 2010 Transferred to Rehab Unit

8 August 2010

Due for discharge home – appointment made for 22 August 2010 for removal of cast
Letter to carer/daughter (NB: heparin to be continued only until mobile)

The patient is being discharged to the care of her daughter.

Writing task:

Using the information in the case notes, write a letter to the daughter, Mrs Holly Kerr, 3 Rose Avenue, Springbank, outlining her mother's medication regime, any potential adverse effects to be aware of, and when to seek medical advice.

In your answer:

- **expand the relevant notes into complete sentences**
- **do not use note form**
- **use letter format**

The body of the letter should be approximately 180-200 words.

Occupational English Test

Pharmacy Letter 3

WRITING SUB-TEST: PHARMACY

TIME ALLOWED: READING TIME: 5 MINUTES

WRITING TIME: 40 MINUTES

Read the case notes and complete the writing task which follows.

Notes:

You are a pharmacist in a community practice. Recently (3 February 2010) you dispensed a prescription for 'Drug X' for Mrs Ellen White.

Mrs White brought the medication back to the pharmacy shortly after it was dispensed because it was out of date. The manufacturer's expiry date on the cardboard carton was 'December 2009'. You apologised, and immediately replaced it with current stock from your shelves. However, Mrs White said that she would complain to the Pharmacy Board about the incident.

The Pharmacy Board is the authority which registers and administers affairs regarding pharmacists in the state in which you live. Its role is to ensure safety and fairness for the public. Complaints against pharmacists are directed to the Board.

In your logbook you wrote these notes for 3/2/10.

New shop assistant (Sarah) started today.

v. busy day too (change in weather perhaps)

I forgot to check expiry date on tablets dispensed to a customer!

Must set up system asap to make sure this doesn't happen again:

- regular checking of expiry dates on all stock (every month)*
- marking anything with approaching expiry date clearly (red sticker?)*
- to remind staff on duty to check carefully*

Writing task:

Write a letter to the Board. Include a response to each item the Board has requested information about. Address your letter to Ms Anne Seaborn, Director, Pharmacy Board, Newtown.

In your answer:

- **Expand the relevant notes into complete sentences**
- **Do not use note form**
- **Use letter format**

The body of the letter should be approximately 180 - 200 words.

Pharmacy Letter 4

	Time allowed:
Reading Time	: 05 Minutes
Writing Time	: 40 Minutes

Read the case notes and complete the writing task which follows.

Case Notes:

An elderly patient has been admitted and diagnosed as having an acute cerebral vascular problem. After all the treatment, patient is showing progression and he is fit for discharge. So here, you are a pharmacist at Green Lane Hospital and you are writing a letter to his wife to ensure the medication regime is followed when he returns home.

Patient History:

Name: Mr Charles Britto

Date of Birth: 10 March 1934

Allergies: Shellfish

Current Medication:

On Admission:

Aspirin 325 mg (Antiplatelet) : 325mg OD at 10:00am

Paroxetine : 12.5mg OD at 2:00pm (Anti depressant, has a history of depression)

On Discharge:

Aspirin 325 mg (Anti platelet) : 325mg OD at 10:00am

Paroxetine : 12.5mg OD at 2:00pm (Anti depressant, has a history of depression)

Taxim: 500mg TDS for 7 days

Zantac (ranitidine) (for gastric upset): 150mg bd for 7 days

Atorvastatin: 10mg OD at night

Heparin low molecular weight (LMWH) (anti-coagulant): 7500 bd – to be continued till next visit

Durolax (to prevent constipation): 10mg at night

Drug Information: Adverse Drug Reactions

Aspirin Adverse: Bleeding, GI disturbances

Ranitidine Adverse: headache; GI upset; rash; CNS disturbances (rare)

Atorvastatin Adverse: Serious: rhabdomyolysis, myopathy, myalgia (0.2%); GI upset (1%); headache (2%); rash (2.5%); flu-like symptoms (1.5%); raised LFTs (1.3%)

Heparin Adverse: haemorrhage, easy bruising, nausea, vomiting

Taxim Adverse: Rare: sensitivity reactions; nausea, vomiting, mild diarrhoea; allergic skin rash or hives

Social History:

Patient lives with his wife. All their children are away. They both live alone.

He is a smoker and an alcoholic.

Height 160cm; Weight 85 kg

Dentures –Nil

Depression-controlled by medication

17 MARCH 2013

10:00 am, Patient brought to EMD in a car with his wife.

Presenting symptoms:

Britto's wife found him lying on the floor confused and soaked in urine.

10:15 am: Patient was seen by Dr Green.

CT Skull scan was done and then MRI was prescribed.

11:30am: Transferred to ward

All the routine investigations ordered:

LFTs, platelet count, WBC count, WBC types, RBC count, RBC indices, Hg, haematocrit, blood smear, ECG & chest X-ray

Advise:

- care of pressure areas.
- fluid balance IV fluids.
- nutritional management: according to dietician's advice.
- thrombus prevention: low dose, low molecular weight heparin, & mechanised compression stockings

28 MARCH 2013 Discharge**Writing task:**

Using the information in the case notes, write a letter to her wife, White Building, Thames Park, outlining her husband's medication regime, any potential adverse effects to be aware of, and when to seek medical advice.

In your answer:

- Expand the relevant notes into complete sentences
- Do not use note form
- Use letter format

The body of the letter should be approximately 180-200 words.

END OF WRITING TEST

Pharmacy Letter 5

	Time allowed:
Reading Time	: 05 Minutes
Writing Time	: 40 Minutes

Read the case notes and complete the writing task which follows.

Case Notes:

Mrs. K Katherine is in her 40's and has been suffering from thyroid related problems. A woman living next to her door brings a prescription for you to dispense. You notice that the medication is for the treatment of common arthritis. As per your records, Mrs. K Katherine is not taking any medication related to joint pains or any other anti-rheumatic medication.

Prescription:

Dr Tobby Perera, 2/249 Darlinghurst.
Phone: +61 2 8084 7822
Mrs K Katherine,
140 Parramatta Rd Ashfield.

30-minute intravenous (IV) infusion (X)
Dosage after every four weeks for three months

Dosing:

There is no need to fast or avoid any particular foods before you start this anti rheumatic infusion.

This anti-rheumatic drug is a 30-minute intravenous (IV) infusion First dose is always followed by a second dose around day 15 and a third dose around day 30.

The patient will then have to continue taking one dose every 4 weeks thereafter.

Possible effects:

Common side effects include: headache, nausea, soreness in throat, upper respiratory tract infection.

Serious infections: It can make patients more likely to get infections or make the infection that the patient has much more severe. There is a need to seek medical help if any of the following signs of infection occur: fever, feel flu-like, fatigue or weakness, cough, red or painful skin.

Allergic Reactions: Allergic reactions may include: swollen face, swollen eyelids or lips or tongue, trouble in breathing is also noted. Known to increase Hepatitis B viral infection, slow down the action of vaccines, certain kinds of cancer have also been reported in patients who take this anti rheumatic drug.

Indication and Usage:

It reduces signs and symptoms in almost all of the patients suffering with moderate to severe rheumatoid arthritis. It prevents damage to bones and joints and effectively helps patients in performing their day-to-day activities.

Writing task:

Using the information in the case notes, write a letter to Mrs. K Katherine, 140 Parramatta Rd Ashfield, outlining its use, any potential adverse effects to be aware of, and when to seek medical advice.

In your answer:

- Expand the relevant notes into complete sentences
- Do not use note form
- Use letter format

The body of the letter should be approximately 180-200 words.

END OF WRITING TEST

Occupational English Test

WRITING SUB-TEST: PHARMACY

TIME ALLOWED: READING TIME: 5 MINUTES

WRITING TIME: 40 MINUTES

Pharmacy Letter 6

Read the case notes and complete the writing task which follows.

Notes:

Today, 1 March 2010, Mrs Daniels, a 78-year-old customer, has come to your community pharmacy asking for something to treat an itchy rash which has been troubling her for the past few days. The rash is quite evident, consisting of large, raised areas which are not red, except where Mrs Daniels has recently scratched them. The rash is mainly on her trunk with some involvement of the upper arms and legs. Mrs Daniels mentions that she has not been gardening, is not prone to rashes, and has no idea about the cause of the rash. When questioned, she says that she has not changed her diet or used different washing powders, soaps, deodorants or perfumes in the last few months.

You note that Mrs Daniels started taking a recently released ACE inhibitor, 'Drug X', about two weeks ago after suffering side effects with verapamil, which she was taking for hypertension.

Medication history for Mrs Daniels:

17/02/10	'Drug X'	5mg	1bd
11/02/10	Indapamide	2.5mg	1d
26/01/10	Verapamil	80mg	1bd
09/01/10	Nitrazepam	5mg	1n

PRODUCT INFORMATION FOR 'DRUG X'

Description: 'Drug X' is a highly specific competitive inhibitor of angiotensin I converting enzyme, the enzyme responsible for converting angiotensin I to angiotensin II.

Pharmacology: The mechanism of action of 'Drug X' has not yet been fully elucidated; however, it appears to lower blood pressure through suppression of the renin-angiotensin-aldosterone system. This results in a decrease in angiotensin II, subsequent to ACE inhibition, and an increase in plasma renin activity.

Indications:

- Hypertension
- Cardiac failure

Warnings:

- Proteinuria has been reported in a small number of patients with pre-existing renal impairment.
- Neutropenia/Agranulocytosis has occurred in a small number of patients.
- Hyperkalemia may occur – potassium supplements and potassium diuretics must be used with caution.
- Cough – a persistent, non-productive cough reported in some patients.

Precautions:

- Hypotension may occur occasionally within first three days of therapy.
- Hypersensitivity reaction – rashes, often associated with pruritis and occasionally fever, may occur.
- Impaired renal function – increases in BUN and serum creatinine have occurred during treatment.

Adverse Reactions: More common reactions:

- Hypotension.
- Dermatological – Rash occurred in 10% of patients. Usually pruritic and macro-papular, but rarely urticarial. Generally occurs during the first four weeks of treatment. Usually self-limited and reversible and may respond to antihistamine therapy.
- Gastrointestinal – A small number of patients reported loss of taste sensation.

Less common reactions:

- Cardiovascular – Tachycardia, chest pain.
- Dermatological – Angiodema of the face.
- Gastrointestinal – Nausea, vomiting, pain and constipation may occur.
- Other – Paraesthesia of the hands.

Presentation: 5mg & 10mg oral tablets

Task 1 suspect an ADR (adverse drug reaction) to the ACE inhibitor and decide on the following course of action:

- Advise Mrs Daniels to visit her doctor as soon as possible.
- Notify Mrs Daniels' doctor, Dr J Sottio, of Newtown.
- Provide immediate treatment with half percent hydrocortisone cream.
- Report the suspected ADR to the relevant authority: the Adverse Drug Reactions Data Bank, PO Box 112, Centreville.

Writing task:

Write a letter to the Registrar of the Adverse Drug Reactions Data Bank, reporting the suspected ADR and all relevant information.

For your answer:

- Expand the relevant notes into complete sentences
- Do not use note form
- Use letter format

The body of the letter should be approximately 180 - 200 words.

Pharmacy Letter 7

Sample Writing Test - Pharmacists

OCCUPATIONAL ENGLISH TEST

WRITING TEST: PHARMACISTS

Time allowed: 40 minutes

Patient

Mrs Charlton, in her late 60's, is a regular customer of yours. Her neighbour brings in a prescription for you to dispense. You recognise that the medication is for treatment of hypertension. According to your records, Mrs Charlton is not currently taking any medication and has not previously taken this or any other anti-hypertensive medication.

Prescription

Mrs V Charlton
16 White Street
Crystal Creek, 3111

Dr B Goodrich
234 High Street
Crystal Creek, 3111
Ph : 802 9743

Drug X 5mg
30 tabs
Sig: 1 mane pc

Thank You

Prescribing information for drug X

Actions:

Antihypertensive with a 24 hour action.

Indications for use:

As a primary measure in the treatment of mild to moderate hypertension and as an adjunct to other antihypertensive agents in the treatment of severe hypertension.

Contra-indications:

Known sensitivity to the drug or others of its class. Drug X should not be used for patients with severe renal disease or complete renal shutdown or in patients with severe liver disease and/or impending hepatic coma.

Precautions:

All patients should be observed for clinical signs of fluid or electrolyte imbalance, including hyponatraemia. These include thirst, dryness of the mouth, lethargy and drowsiness. With intensive or prolonged therapy it is important to guard against hypochloraemic alkalosis and hypokalaemia.

Sample Writing Test - Pharmacists

Adverse Reactions:

Gastrointestinal: anorexia, gastric irritation, nausea, vomiting, cramping, diarrhoea, constipation, jaundice, pancreatitis.

C.N.S.: dizziness, vertigo, paresthesias, headache, xanthopsia

Cardiovascular: orthostatic hypotension

Haematological: leucopenia, agranulocytosis, thrombocytopenia, aplastic anaemia.

Hypersensitivity (dermatological): purpura, photosensitivity, rash, urticaria, vasculitis.

Other: hyperglycaemia, glucosuria, hyperuricemia, muscle spasm, weakness, restlessness.

Impotence has been observed with some drugs in the group when used in high dosages.

Interaction:

The following have been reported:

Alcohol & barbiturates: orthostatic hypotension may occur or be aggravated;

Digitalis: increase digitalis toxicity;

Ganglionic and peripheral adrenergic blocking drugs: potentiation of effect;

Insulin: diabetic control may be altered;

Lithium: lithium toxicity increased;

Muscle relaxants: neuromuscular block increased;

Noradrenaline: decreased arterial responsiveness;

Oral antidiabetic agents: reduced effectiveness;

Phenothiazines: shock

Overdosage:

Symptoms: Symptoms include electrolyte imbalance and signs of potassium deficiency such as confusion, dizziness, muscular weakness and gastrointestinal disturbances.

Treatment: General supportive measures, including replacement of fluids and electrolytes are indicated.

Use in pregnancy and lactation:

Pregnancy Category C.

Drug X should be used with caution by pregnant women and by nursing mothers since drugs in the group cross the placental barrier and appear in cord blood. Use may result in foetal or neonatal jaundice, bone marrow depression and thrombocytopenia, altered carbohydrate metabolism. In newborn infants of mothers showing decreased glucose tolerance, and other adverse reactions which have occurred in the adult; when the drug is used in pregnant women, the potential benefits of the drug should be weighed against the possible hazards to the foetus.

Australian categorisation definition:

Category C: Drugs which, owing to their pharmacological effects, have caused or may be suspected of causing, harmful effects on the human foetus or neonate without causing malformations. These effects may be reversible.

Dosage and Administration:

Administered orally

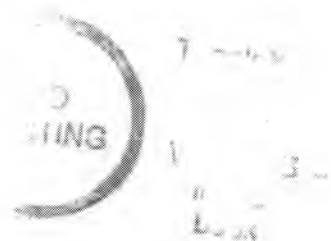
Adults: Usual dose: 2.5 to 10 mg once daily. To maintain an oedema-free state or as adjunct in the management of hypertension, 2.5 to 5 mg once daily. Usual optimum daily dose: 5 mg, maximum effective single dose is 10 mg. Administer after food to minimise gastrointestinal side effects. Monitor for dizziness after initial dosage.

In the treatment of hypertension, Drug X may be either employed alone or concurrently with other antihypertensive with lower dosage of the component drugs and fewer or less severe side effects.

Writing Task

Using the drug information provided, write a letter to Mrs Chariton summarising advice on how to take her medication, what side-effects to be aware of and how to cope with them.

Do not use note form in the letter. Expand the relevant case notes into full sentences. The letter should be 15-20 lines long. If it is longer only the first 25 lines will be assessed.



PHYSIOTHERAPY

Physiotherapy Letter 1

Writing Task Sample – Physiotherapy

Time allowed: 40 minutes

Read the cases notes below and complete the writing task which follows:

Today's Date

12 February 2010

Patient History:

Surname Stewart
Given Names Anthony
Birth date 23.10.64
Occupation National Park Ranger
Social Married with 2 teenage children -works full time
Diagnosis Talar dome cartilage deficit in right ankle
CT Report (27.1.10) no abnormality detected in bones

Past History:

Jumping off from a 1.5metre height of fence at work, twisted ankle badly on 03.11.09
Referred by GP Dr. David Robertson for physiotherapy
Occupational activities: walks in rough terrain every day
Sports: surfing, soccer –social game every Saturday, coaching his teenage son. Recurrent ankle sprain bilaterally when played in local club 10 years ago

17. 11.09

Assessment

Walking with a pair of crutches
Moderate swelling and bruise around right ankle and dorsum of foot
Restricted movement: DF: -5 degrees, PF: 10 degrees, inversion: eversion = 6:1 (limited eversion).
Foot to wall: -2cm (right) vs 10 cm (left) (normal:12-14 cm)
Anterior draw and Talar tilt: unable to test due to pain

Treatment

Ultrasound
Taping
A home based exercise program: stretches with towel, ankle pumps

Plan

Review in 3 days

20.11.09

Assessment

Improved
Mild swelling and bruise
DF: 0 degree
Foot to wall: 0cm (right)

Treatment

Ultrasound
Taping
Taught to walk with one crutch
Stretches of gastrocnemius and soleus

Plan

Review in 3 days

This resource was developed by OET Online

22.12.09	(4 weeks later after 8 treatment sessions) No new complaint
Assessment	DF: 8cm (right) vs 10cm (left)
Treatment	Ankle guard Functional exercises: lunges, jogging, step ups Will go away for Christmas holidays for 4 weeks Provided a home exercise program include stretches, strengthening, balance training and functional tolerances training
Plan	Review after his holiday
24.01.10	Pain after surfing, pain was aggravated after walking even wore the ankle guard
Assessment	Mild intra-articular effusion DF: 0 degree PF: 5 degrees Foot to wall: 2cm (right) vs 10cm TOP (tenderness on palpation): medial joint line and talar dome Anterior draw: no laxity in ATFL
Treatment	Taping Stretches Grade I joint mobilisation
Plan	Refer to see GP
12.02.10	Pain after joint mobilisation CT result was back
Plan	Referral to his GP: Dr David Robertson for orthopaedic opinion – MRI to rule out a cartilage deficit of talocrual joint or talar dome fracture, or arthroscopy.

WRITING TASK

Using the information in the case notes write a letter of referral to Dr David Robertson, General Practitioner, 115 King Street, Warners Bay, 2284

In your answer

- 1 Expand the relevant case notes into complete sentences*
- 2 Do not use note form*
- 3 Use correct letter format.*

The body of your letter should be approximately 180 - 200 words.

OCCUPATIONAL ENGLISH TEST

WRITING SUB-TEST: PHYSIOTHERAPY

TIME ALLOWED: READING TIME: 5 MINUTES

WRITING TIME: 40 MINUTES

Read the case notes and complete the writing task which follows.

Notes:

You are a physiotherapist in private practice. Max Wolff has been referred to you by his GP, Dr William Stacey, for review and a treatment plan after presenting with chronic back pain.

Physiotherapy Notes – from initial consultation 1 May 2010

Personal Details:

Name: Max Wolff (Mr)
Age: 35
Profession: Full-time musician: orchestral double bass player
Lives with spouse, also a musician
No dependants

Family/Patient History:

Father (70) has mild osteoarthritis; mother (67) healthy
Younger brother & sister healthy

Tonsillectomy/adenoidectomy (1979)
Myopic (corrective lenses since age 14)

Non-smoker; 'social' drinker (8-10 units/week)

Mild idiopathic scoliosis (<20°, untreated) since teenage years: slouching at desk while studying at school & music college

Minor, ongoing postural problems from music college to present: daily work routine (practising, attending rehearsals & performing with orchestra); pain not a problem until recently

Little formal exercise (no sports, no gym); busy schedule, with frequent evening work

Subjective: Pt complains of ongoing upper back pain – feels stiff, 'frozen', 'locked' between shoulders; also dull pain in lumbar region

Agg: prolonged performance on instrument (>2 hrs); ease: rest

Symptoms developing over last 6-10 months; pt too busy at work to attend GP; has been using non-prescription analgesics lately for relief (to help with sleep, esp. after evening performances)

Bass playing requires particular body posture – pt normally sits on high stool with body weight mainly on R leg; L arm is bent & raised up to near pt's ear on instrument, R arm reaches forward to produce sound with bow. Unbalanced posture.

Pt concerned that current symptoms may prevent participation in important international tour with orchestra (for 1 month, leaving in 3 weeks) – this was trigger to attend GP.

Also aware, however, of need to find & treat cause of current symptoms to maintain long-term health & continued capacity to perform (= earn).

Physical Examination Findings:

Standing posture – mild thoracic kyphosis with protraction of both scapulae & forward head posture. Average build with lax abdominal muscles.

Flexion in standing – fingertips 10cm below knees, mild scoliosis convex on right.

Extension in standing – stiff ++

Side flexion in standing – fingertips to knee on left – complains of right lumbar tightness; fingertips 5cm above knee on right with stiff segment T3-T8.

Spinal rotation in sitting – stiff end of range to left but range normal. Pain reproduced with overpressure; ¾ range to right – stiff segment T3-T8.

Palpation – increased tone & tenderness left erector spinae T6-T8 & right erector spinae L2-L4. Stiff PA central & right unilateral T3-T8.

Treatment Plan:

Posture training including cross-tape to mid thoracic spine to promote postural awareness & self-correction of forward posture.

Soft tissue releases left erector spinae T6-T8 & right erector spinae L2-L4.

Spinal mobilisation T3-T8 to increase extension & right rotation.

Home exercises: Right side flexion in sitting bringing left arm over head; right rotation in sitting with hands behind neck, elbows forward – eight repetitions of each exercise with 10 second stretch at end of range – repeat four times each day.

Review twice each week until departure – introduce strength exercises & self-massage using tennis ball at next session. Advised patient that problem is not acute – should be able to participate in tour but will need to exercise, do self massage & use tape for posture while away.

Writing task:

Using the information in the notes, write a letter back to the referring GP detailing your findings and suggested treatment plan. Address your letter to Dr William Stacey, Greywalls Clinic, 23 Station Road, Greywalls.

In your answer:

- **expand the relevant notes into complete sentences**
- **do not use note form**
- **use letter format**

The body of the letter should be approximately 180-200 words.

Physiotherapy Letter 3

OET WRITING - PHYSIOTHERAPISTS

You are a Physiotherapist at the Cabrini Rehabilitation Centre, Kew, Victoria.

Patient History

Brad Johnston 78 years old
Widower; lives by himself in a town house, 122 Clara St. Fawkner
Used to work as a plumber until the age of 65
Was a heavy drinker until age of 58, used to play basketball, cricket
and swimming at different stages of his life

Diagnosis

-CVU (Cardio vascular attack) on 07-Jan-09 resulted in left hemiplegia
-C.T scan showed a moderate hemorrhage in frontal and parietal areas of the brain

History

High blood pressure since 1982; diabetes since he was 50;
laser eye correction in 1998; Right knee osteoarthritis since 1976

Notes

May 4, 2010 Started Passive R.O.M exercises for left upper and lower extremities and PNF (Proprioceptive Neuromuscular Facilitation) technique.

From first day pain at the beginning of the exercises and end of range of motion; patient was resistant to commence any exercises and did not want to co-operate; was referred to a psychiatrist for counselling / treatment.

May 13, 2010 Patient able to walk independently assisted by walking frame (100 meters once a day). Also doing mobilizing exercises. ROM and muscle strength have improved. Patient is ready for discharge.

TO BE REFERRED ON TO LOCAL PHYSIOTHERAPIST 14 MAY 2010

TASK

Using the information in the case notes, write a letter of referral to:

Ms Janet Stevens Physiotherapist Fawkner Rehabilitation Centre 1255 Hume Highway, Fawkner, Vic. 3060 outlining a suitable physio regime for Mr Johnston during the next three months.

Write in complete sentences. Your letter should be no more than 180-200 words.

Physiotherapy Letter 4

OET WRITING - PHYSIOTHERAPIST

Read the case notes below and complete the writing task that follows.

The patient wishes to return home after staying with his daughter, he will attend a local private practitioner,

Patient History:

Surname: Taylor
Given Names: Tom
Age: 74 years
Occupation: Retired

Referral:

Fractured lower 1/3 of left tibia and fibula 4 months ago, Partial weight bearing for 2 weeks then progress to full weight bearing. Review /X-ray 15.11.91

X-Ray report 11/6/91

An oblique fracture of the distal 1/2 of left tibial shaft and a fracture of upper 1/3 of the left fibula shaft is in satisfactory position

Early osteoarthritis of the left knee joint is noted

15.10.91

Patient fell 3 feet off a ladder in the house, fracturing left tibia and fibula

Reduced under local anesthetic

Above knee P.O.P x 9 weeks, then below knee P.O.P x 9 weeks

Removed yesterday

Belongs to walking club; keen gardener

Obs: Moderate swelling of lower leg
Petting edema of foot to 3cm above ankle

Dry, flaky skin

Quads/calf wasting

P.W.B on crutches

R.O.M. Dorsiflexion = +2 degrees (R=12 degrees)
Plantarflexion = 25 degrees (R= 50 degrees)
Inversion = 1/8
Eversion = 1/2
L Toes 3 L knee 3

Treatment

Home exercise programs (quads over fulcrum x 20. calf rubber x 20, in – and eversion with towel x 10, foot circling x 20, active plantarflexion x 20)

Exercise card given

19.10.91

Some sharp jobs of pain in the leg, tubigrip too tight

Obs

DF = +5 degrees

PF = 40 degrees

INV = 2/3

EV = ½

Treatment

Revise exercise program. Add exercises in sitting-foot sliding x 10, toe/heel praises x10

New tubigrip

Requests referral to private practitioner nearer his own home

Writing Task

Using the information in the case notes write a letter of referral to Miss Louise Johnston, Physiotherapist, 25 Main Rd, Preston

Physiotherapy Letter 5

OET WRITING – PHYSIOTHERAPISTS

Read the case notes below and complete the writing task which follows:

- Patient to be discharged from Heidelberg Rehabilitation hospital today, following a work accident.

Patient's details:

- Evan MILLAR d.o.b. 14 July 1980
- Forklift driver, Warehouse
- Lives with a flatmate - Single

18 Jan 2009

- Admitted to Royal Melbourne Hospital A & E
- Had sustained a work accident – crushed under a forklift truck

20 Feb 2009

- Discharged to Heidelberg Rehab Hospital

21 Feb 2009 – Physiotherapist's assessment

Cognitive Memory difficulties; blurred vision; loses balance with ambient distraction; distracted by auditory and visual stimuli

Physical Ambulant, balance disturbances; R side weakness; R side facial numbness; R dominant – unable to wink; tightening of R forearm and hand; slow fine motor activity

Care plan: Improve balance and ambulation; improve fine and gross Upper extremity function; increase fitness and improve sleep patterns

Therapy Myofascial release and movement (R arm function);

Plan Cranial sacral therapy [sleep]; fine motor speed and visual-spatial skills incorporating movement.

25 Mar 2009 Physiotherapist's assessment: Improved balance, patient walks outside; reduced tightness in R hand.

Plan Increase fitness and stamina; introduction of keyboard; Medication for sleep to be phased out

Discharge Conference with speech therapist and O.T. and medical

Plan staff prior to discharge; refer to community physio for weekly, then monthly treatment; to live with parents; patient wishes to resume driving.

Writing task:

Using the information in the case notes, write a letter of referral to: Mr Johnny Ramone, The Heidelberg Physiotherapy Centre, Brick Road, Heidelberg Vic 3084. In your answer:

- expand the relevant case notes into sentences
- do not use note form
- Body of letter should be approx 200 words
- Use correct letter format

Physiotherapy Letter 6

MATERIALS

Writing Test – Physiotherapists

Time allowed: 40 minutes

Read the case notes below and complete the writing task which follows.

The patient is to be discharged from the orthopaedic ward to a rehabilitation centre where he will attend as an outpatient.

Patient history Surname: Browning

Given Names: John Louis

Birthdate: 30.10.39

Occupation: Credit Manager

Social: Lives with his wife. Children have moved out.

Diagnosis: Elective total knee replacement on 16.12.96

X-ray Report (19.12.96): L Total Knee Replacement position appears satisfactory

Past history L Knee trouble for many years – osteoarthritis, instability, intermittent locking. Painful most of the time. Uses a walking stick. Was an A-grade soccer player. Years of knee pain L > R Keen sportsman in the past. Previously independent.

17.12.96 Resting in bed with a zimmer knee splint.

Treatment Deep breathing and coughing exercises

Bed exercises: static quads, straight-leg raise, foot and ankle

Plan Continue bed exercises, mobilise when able, aim for home

18.12.96 Complaining of pain

Treatment Continue bed exercises

Poor static quadriceps contraction – unable to lift leg

Plan To commence ambulating on Friday

20.12.96 Pain decreased

Treatment Bed exercises as previously – still not able to straight leg raise

Quad exercises ++

Commence active knee fl exion = 30°

Commence partial weight bearing with crutches and Zimmer splint – walked 10 metres with diffi culty

24.12.96 No change in range of motion or quads strength

Continue bed exercises and walking

Encourage ++

4.1.97 No change. For manipulation under anaesthetic tomorrow.

6.1.97 Having intensive physiotherapy

Knee fl exion = 60°

Quads lag – 10°

Walking independently between crutches

Refer to rehabilitation centre for out-patient physiotherapy

Review in out-patient clinic: 6.2.97

Writing Task

Using the information in the case notes, write a letter of referral to Ms Barbara Blunt, Physiotherapy Department, St Stephen's Rehabilitation Centre, Bond Street, Burwood, 3125.

In your answer:

- Expand the relevant case notes into complete sentences.
- Do not use note form.
- The body of the letter should be approximately 200 words.

NURSING

Nursing Letter 1

Writing Test

Time allowed:
Writing : 40 Minutes

Read the case notes below and complete the writing task which follows.

Hospital	Royal Perth Hospital
Patient Details	Alfred Billy 52 Years old Marital status: married Wife to be contacted if there is any sort of emergency: Maria Jennifer, Arillon City Arcade 207 Murray Street Perth
Admission Date	21/03/2010
Discharge Date	5/05/2010
Diagnosis	Skin cancer – BCC (Basal Cell Carcinoma) (neck) Nodular basal-cell carcinoma
Past Medical	No prior hospitalization, no history
History	Medications
Social	Truck Driver
History/Supports	Lives with her wife Habit of consuming liquor for th past 30 years Cigarette Smoker Skin dark Religion: Protestant
Medical Progress	Skin biopsy is taken for pathological study CCB - removal of Pain reliever panadein forte 500mg
Nursing	No complications noted
Management	Perfectly well at the time of discharge No complain of any pain

Discharge Plan

Daily obs

Medicine to be taken for one more week

Writing Task

You are the charge nurse on the hospital ward where Mr. Alfred Billy has recently had his operation. Using the information provided in the case notes, write a referral letter to the Community Nurse Head at Care Well Hospital, Birmingham, who will be attending to Mr. Alfred Billy, following his discharge.

In your answer:

- Expand the relevant case notes into complete sentences.
- Do not use note form.
- The body of the letter should be approximately 200 words.
- Use correct letter format.

Writing Test: Nurses

Time allowed: 40 minutes

Read the case notes below and complete the writing task which follows.

Notes:

Ms. Amy Vineyard is a patient in your care at the St Kilda Women's Refuge Centre. She is 6 weeks pregnant with her first child. She presented two days ago, requesting help for her substance abuse problems. She reports a desire to reduce or cease her alcohol consumption and a desire to reduce or cease her drug use. No desire has been indicated to decrease or stop cigarette use. She now wishes to be discharged but will require ongoing support throughout her pregnancy.

Discharge summary:

Name: Ms. Amy Vineyard

Age: 21

Admission: 6/1/09

Diagnosis: pregnant substance abuse

Discharge: 8/1/09

Plan:

- Community mental Health Nursing required daily next 2 weeks minimum.
- Pt wishes to continue living with a friend on her sofa.
- Psychiatric support needed for depression.
- Methadone program Alcoholics Anonymous meetings
- 1 Trimester Ultrasound at 2 weeks;
- maternal health clinic appointment needed.

Reason for admission:

- Pt. self admitted due to concern about pregnancy. Confirmed pregnancy test the days before (5/1/09)
- Reported pain in lower back
- weight loss (6kg over 2 months)

- some memory loss
- tingling in feet, difficulty sleeping, excessive worry and hallucinations
- feeling depressed-history of depression
- no pain in hips or joints
- no decrease in appetite
- no double vision

Treatment

- pt. monitored and blood tests for HIV/AIDS and STDs
- counseled re nutrition and pregnancy
- counseled re HIV/AIDS and STDs risk
- discussed possibility of rehabilitation clinic for 'driving out'

Lifestyle:

- Nicotine daily 30-40 cigarettes
- started smoking at 15 y. o.
- Drugs used cannabis, amphetamines, cocaine, heroin
- started all above at 16 y. o.
- injects heroin, occasionally shares infecting equipment
- Alcohol 8 units/day ___ max. units/day- 15
- started drinking at 16 y. o.
- lives with a friend, Sophie, on her sofa.
- no contact with parents

History:

- suicidal thoughts, self harm in past
- never seen a psychiatrist

Writing Task

Using the notes, write a letter about Ms. Vineyard's situation and history to new community health nurse. Address your letter to Ms. Lucy Wan, Registered Nurse, Community Health Centre, St Kilda.

Nursing Letter 3 Sample Writing Task: Nurse

Time allowed: 40 minutes

Read the case notes below and complete the writing task which follows:

You are Sonya Matthews, a qualified nursing sister working with the Blue Nursing Home Care Agency. Bob Dawson is a patient in your care. Read the case notes below and complete the writing task which follows.

Name: Bob Dawson
Address: 141 Montague, West End 4101
Phone: (07) 3442 1958
Date of Birth: 25 September 1924

Social Background

Married - wife Elizabeth aged 83. Lives in own home - Both receive age pensions

Bob is World War11 Veteran with Gold Health Card entitlement

Medical History:

Cerebrovascular accident (CVA) 4 years ago

Rehabilitation generally successful - Mentally alert, slight speech impairment, - residual weakness left side - walks with limp - balance slightly impaired.

18 /5/08

Had fall descending stairs. Badly grazed left knee. GP has requested daily visits by Blue Nursing Home Care to dress wound and assist with showering.

19.5.08

Grazed knee redressed - no sign of infection

Bob managing to get around the house slowly with aid of his wife.

Reports that apart from "usual aches and pains" he is doing well.

23.5.08

Knee healing well.

Suggested use of a walker or walking stick to assist with mobility.

Bob said he had a walking stick but it was useless. Wife says he had never learned to use it properly. She asked if I would contact their local physiotherapist to see if Bob could receive a home visit to assess further assistance to improve his mobility.

WRITING TASK

Using the information in the case notes, write a letter to Ms Marcia Devonport, West End Physiotherapy Centre, 62 Vulture Street, West End, Brisbane 4101 on behalf of Mrs Elizabeth Dawson requesting a home visit to provide advice and assistance with improving her husband's mobility. Do not use note form in the letter. Expand on the relevant case notes to explain his background and medical history and the assistance requested. The letter should be 15-20 lines long. No more than the first 25 lines will be assessed.

Nursing Letter 4

Sample Writing Task: Nurse

Time allowed: 40 minutes

Read the case notes below and complete the writing task which follows:

You are Sonya Matthews, a qualified nursing sister working with the Blue Nursing Home Care Agency. Bob Dawson is a patient in your care. Read the case notes below and complete the writing task which follows.

Name: Bob Dawson
Address: 141 Montague, West End 4101
Phone: (07) 3442 1958
Date of Birth: 25 September 1924

Social Background

Married - wife Elizabeth aged 83. Lives in own home - Both receive age pensions

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Had fall descending stairs. Badly grazed left knee. GP has requested daily visits by Blue Nursing Home Care to dress wound and assist with showering.

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Reports that apart from "usual aches and pains" he is doing well.

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Bob said he had a walking stick but it was useless. Wife says he had never learned to use it properly. She asked if I would contact their local physiotherapist to see if Bob could receive a home visit to assess further assistance to improve his mobility.

WRITING TASK

Using the information in the case notes, write a letter to Ms Marcia Devonport, West End Physiotherapy Centre, 62 Vulture Street, West End, Brisbane 4101 on behalf of Mrs Elizabeth Dawson requesting a home visit to provide advice and assistance with improving her husband's mobility. Do not use note form in the letter. Expand on the relevant case notes to explain his background and medical history and the assistance requested. The letter should be 15-20 lines long. No more than the first 25 lines will be assessed.

Nursing Letter 5

Mavis Brampton [5 mins reading / 40 mins writing]

This patient has been in your care and is now going home from the Northern Community Hospital, Moreland, 3051.

Patient: MAVIS BRAMPTON - 72 years old

Admitted: 10 January 2011 To be discharged: 15 January 2011

Diagnosis: Pleurisy

BACKGROUND:

□ Mrs Brampton has been widowed 25 years. Has been an active member of the community all her life. Is the current President of PROBUS in her area. She with her husband ran the Sydney Road Newsagency until his death at which time she retired. Attends the local Community Centre three times a week to play Bingo. Has been a smoker all her life (since 18 years of age). Current smoking 10 a day.

NURSING NOTES:

- 10 Jan 2011 Overweight: BMI 29 Had CXR; IV Amoxycillin with supplementary O₂
- Advised to give up smoking.
- BP 170/90 Pulse 92 Slightly raised temperature: 39°C Breathless
- 12 Jan 2011 On low-dairy diet Advised about Nicotine patches.
- Productive cough – sputum culture done Pravastatin 20mg/day and Celecoxib 100mg/day

13 Jan 2011

- Deep breathing exercises started. Is keeping to a non-smoking regime.
- Using Nicotine patches and Zyban (150mg b.i.d).
- To be discharged 15 Jan 2011.

DISCHARGE PLAN:

- Support Mrs Brampton - needs monitoring for medication compliance
- Needs help with nutritious meals (Meals on Wheels) and house keeping (Council Home Help) - Assistance with shopping
- Monitor her quit-smoking plans - watch for side effects from Zyban such as dry mouth and difficulty in sleeping. If side effects occur Zyban should be stopped. Zyban to be withdrawn after 2 months. Nicotine patches to continue until smoking addiction is under control.

WRITING TASK:

Write a letter of referral to Brunswick Family Care Clinic, 44 Decarle Street, Brunswick, Vic 3056 requesting monitoring and ongoing care be arranged for Mrs Brampton. Community Nurse to make sure Mrs Brampton continues her cessation of smoking – with the help of Nicotine patches and Zyban. Zyban tablets to cease as soon as side effects occur (if any). Both Zyban and Nicotine to cease as soon as craving for cigarettes has stopped. Letter should be 180 to 200 words long / only the first 25 lines will be considered.

Nursing Letter 6

Beverley Williams Born 1943

PATIENT

This patient has been in your care for the past 10 years. During the past 8 years Mrs Williams has developed diabetes. It is not well controlled. You are now referring her on to a Public Health Nurse for a health education program.

HISTORY

- ☐ Type II Non Insulin Dependent Diabetes – onset 8 years ago
- ☐ Prescribed tablets soon after diagnosis
- ☐ No problems with sugars or infections
- ☐ Has monitored urine with sticks at home
- ☐ Not always well controlled
- ☐ Does not care about diet regime
- ☐ High BP for past 5 years – on medication
- ☐ Overweight for past 30 years (BMI 32)
- ☐ Vision OK
- ☐ Has worn spectacles for past 20 years
- ☐ Grandmother had Diabetes; died of gangrene of the foot
- ☐ Husband is also Diabetic

DIABETIC HABITS

- ☐ No special diet
- ☐ Tries not to have sugar
- ☐ Buys diabetic cordial
- ☐ Tastes food while preparing meals in kitchen
- ☐ Eats cream cakes at afternoon tea time
- ☐ Loves fruit
- ☐ Unaware of consequences of careless diet
- ☐ Has trouble losing weight
- ☐ Very little exercise – walks around the neighbourhood occasionally
- ☐ Likes a glass of wine with evening meal

RELATIONSHIPS

- ☐ Has four children – all adults – all married
- ☐ Gets on well with husband
- ☐ Likes visiting her daughter in the country
- ☐ Has active social life – visit friends regularly

TREATMENT PLAN

- ☐ Monitor urine – monitor blood sugar levels with glucometer
- ☐ Needs to be educated re Diabetes and importance of special diet
- ☐ Needs to attend formal diabetic education program (daytime classes at Hospital)
- ☐ Increase Daonil from 15 to 20mg per day
- ☐ Needs vision checked every two to three months
- ☐ Needs to lose weight – has increased 3.5kg in last 6 months
- ☐ Suggest a suitable exercise program ? Swimming

WRITING TASK

Using the information in the case notes, write a letter of referral to: Ms Michella Mansoura, Public Health Nurse, 125 Canterbury Road, Ringwood, Victoria 3134 Australia. DO NOT use note form – use complete sentences. Expand the *relevant* notes in the treatment plan requesting that Ms Mansoura take over the management of this patient. Letter should be no more than 25 lines long.

Dylan Charles

Read the case notes below and complete the writing task that follows.

Time allowed : 40 minutes

You are a Maternal and Child Health Nurse working at the Romaville Community Child Health Service.

Today's date: 15 January 2012

Patient History

- Baby boy: Dylan Charles
- DOB: 04/12/11
- Born: Romaville Maternity Hospital
- First baby of Raymond and Sylvia Charles
- Address: 19 Mayfield St, Romaville
- Discharged 8/12/11

Family History

- Mother: Aged 24 First Child
- Father: Aged 25 Soldier Currently away from home on duty

Birth Histor

- Normal vaginal birth at term
- Birth weight: 3400gm
- Apgar score at 5 min: 9
- No antenatal or postnatal complications

15/01/12 Subjective

- Silvia and baby attended for routine 6 week check-up. Silvia says she is concerned about constipation: once every three days, hard stool. Mother is asking about stool softener or prune juice for baby.
- Breast fed for first three weeks after birth.
- Baby became unsettled during summer heatwave in December.
- Silvia got sick and had a fever for a few days. Mother-in-law (Mary Charles) came to visit and advised changing baby to formula feeds. Mary advised extra powder in formula feeds to improve weight gain.
- Silvia worried she does not have enough breast milk and now gives extra formula feeds as well as breast feeding. Dylan difficult to bottle feed.
- Silvia wishes to breast feed properly as she believes it would be the best thing for her son.
- Mary Charles plans to stay with the family for at least a further month to help with baby. Tensions developing between mother and mother-in-law over what is best feeding method for Dylan.

Objective

- Reflexes normal
- Slightly lethargic
- No abdominal tenderness
- Heart Rate: 174
- Respirations: 56
- Temperature: 37.1
- Weight: 4200gms
- 3 wet nappies in last 24 hours
- Urine dark

Nursing Letter 6

Assessment

- Mild constipation and dehydration

Plan

- Increase breast feeds
- Refer to breast feeding support service
- Check formula is correctly prepared
- If continuing formula feeds, advise to supplement with water (boiled and cooled)
- Advise on keeping baby cool in hot weather
- Return for review in 48 hours.

Writing Task

Please write a referral letter to the Lactation Consultant at the Breast Feeding Support Centre, 68 Main Street, Romaville.

- In your letter expand the relevant case notes into complete sentences
- Do not use note form
- The body of your letter should be approximately 180~200 words
- Use correct letter format.

Nursing Letter 7

Mr Gerald Baker is a 79-year-old patient on the ward of a hospital in which you are Charge Nurse.

Patient Details:

Marital Status: Widower (8 years)
Admission Date: 3 September 2010 (City Hospital)
Discharge Date: 7 September 2010

Diagnosis: Left Total Hip Replacement (THR)
Ongoing high blood pressure

Social Background: Lives at Greywalls Nursing Home (GNH) (4 years)
No children
Employed as a radio engineer until retirement aged 65
Now aged-pensioner
Hobbies: chess, ham radio operator
Sister, Dawn Mason (66), visits regularly; v supportive
– plays chess with Mr Baker on her visits
No signs of dementia observed

Medical Background: 2008 – Osteoarthritis requiring total hip replacement surgery
1989 – Hypertension (ongoing management)
1985 – Colles fracture, ORIF

Medications: Aspirin 100mg mane (recommended post-operatively)
Ramipril 5mg mane
Panadeine Forte (co-codamol) 2 qid prn

Nursing Management and Progress:

daily dressings surgery incision site
Range of motion, stretching and strengthening exercises
Occupational therapy
Staples to be removed in two wks (21/9)
Also, follow-up FBE and UEC tests at City Hospital Clinic

Assessment:

Good mobility post-operation
Weight-bearing with use of wheelie-walker; walks length of ward without difficulty
Post-operative disorientation re time and place during recovery, possibly relating to anaesthetic – continued observation recommended
Dropped Hb post-operatively (to 72) requiring transfusion of 3 units packed red blood cells; Hb stable (112) on discharge – ongoing monitoring required for anaemia

Discharge Plan:

Monitor medications (Panadeine Forte)
Preserve skin integrity
Continue exercise program
Equipment required: wheelie-walker, wedge pillow, toilet raiser. Hospital to provide walker and pillow. Hospital social worker organised 2-wk hire of raiser from local medical supplier.

Writing task:

Using the information in the case notes, write a letter to Ms Samantha Bruin, Senior Nurse at Greywalls Nursing Home, 27 Station Road, Greywalls, who will be responsible for Mr Baker's continued care at the Nursing Home.

In your answer:

- expand the relevant notes into complete sentences
- do not use note form
- use letter format

The body of the letter should be approximately 180-200 words.

Nursing Letter 8

Read the case notes and complete the writing task which follows

Notes

Harry Kovacs is a 5 year old boy who is the son of one of your newly referred patients in the community mental health centre where you are a mental health case manager.

Date of birth: 15 April 2006

Place of birth: Sydney Children's Hospital, Sydney

School year: Kindergarten

Religion & ethnicity: Catholic & both parents Australian born Hungarian

Mother's name: Elizabeth Kovacs

Mother's community admission date: 16 May 2011

Diagnosis: Mother – Major depression with psychotic features

Son – ? Early onset separation anxiety disorder

Family/Psychosocial:

* Elizabeth suffered PND – depressed since

*She sometimes hears voices calling her and sees 'men' running around her house – nil serious psychosis in functional terms.

* Recently 1st psych admission for 6/52 after high lethality DSH attempt.

*Harry's psychological status ok until DSH and hospitalisation; after this +++ signs of separation anxiety

*Father is self employed and works long hours 7/7. Rarely sees Harry & dismissive of Harry's emotional states, 'He's like a bloody girl now!' he told us.

*Harry loves soccer and playing with his dog, 'Rusty'.

Medical History

Eczema

Serous otitis media – required grommets at 18 mths

Hearing NAD now.

Medication

Nil meds

Case management care and progress:

- * Elizabeth new to our area (from Parramatta) & referred to us post D/C from Bankstown MH inpatient unit 2/52 ago
- * We will provide her with long term MH case management.
- * Harry now 1) cries and panics whenever Mum leaves his sight 2) Socially withdrawn & refusing to attend kindergarten 3) ↑ insomnia & nightmares 4) preoccupied re Mum's daily activities & that she might leave him again.
- * This is greatly ↑ pressure on Elizabeth when her MH is already fragile.
- * Father, John, uninterested in meeting in person or discussing problems in detail.
- * Harry attended initial assessment with Elizabeth and separation anxiety behaviour very obvious

Referral plan:

- * Referral to early childhood mental health team for assessment and management of Harry's ? early onset separation anxiety disorder.
- * Request joint meeting with case manager and Elizabeth.

You are the Case Manager caring for Harry Kovac's depressed mother but due to his psychological issues need to write a referral for him to John Dyer, Clinical Psychologist on the Bankstown early childhood mental health team at Bankstown Hospital.

In your answer:

- Expand the relevant notes into complete sentences
- Do not use note form
- Use letter format

The body of the letter should be approximately 180-200 words.

Nursing Letter 9

Time allowed: 40 minutes

Read the case notes below and complete the writing task which follows:

Today's date: 9/7/08

Patient Details

Jim Middleton aged 84 was admitted to your ward following surgery for a left inguinal hernia. His doctor has advised he can be discharged within 48hrs if there are no complications following the surgery. Jim reports some pain on movement but has recovered well from the surgery and is keen to return home.

Name: Jim Middleton

Date of Birth: 3 July 1924

Admitted: 7 July 2008

Planned Discharge Date: 9 July 2008

Diagnosis: Left inguinal hernia

Medical History

Hypertension diagnosed 1998

Medication Atacand 4 mg daily

Family History

Married 50 years to wife Olga DOB 8.2.32 - one son living in USA

Jim is Second World war veteran - served two years in Borneo -Prison of War 16 months.

Own their home with large garden which they maintain without assistance.

Very independent and proud that they have never applied for a pension or home assistance.

Have always managed quite well on their income from a number of investments.

Olga told you she is worried as income from these investments has recently been significantly reduced due to severe stock market falls. She is concerned Jim will not be able to continue to maintain their garden and they will not be able to afford a gardener or any other help at this time.

Transport is also a problem as Olga does not drive. Not close to any public transport so will have to rely on taxis. Olga thinks they may now be eligible to receive a pension and other assistance from the Department of Veteran Affairs but doesn't know how to find out - doesn't want to worry Jim.

Olga is in good general health but becoming increasingly deaf - finds phone conversations difficult. She would appreciate a home visit. You agree to enquire on her behalf. Their address is 22 Alexander Street, Belmont, Brisbane 4153 Phone (07) 6946 5173

Discharge Plan

- Must avoid any heavy lifting
- Should not drive for at least six weeks
- Light exercise only
- May take 2 Panadol six hourly for pain
- Appointment made to see surgeon for post operation check at 10am on 11 August
- Contact Department of Veterans Affairs re eligibility for pension and home help

WRITING TASK

Using the information in the case notes, write a letter to The Director, Department of Veterans Affairs, GPO Box 777 Brisbane 4001. In your letter, explain why you are writing and the assistance they are seeking.

Do not use note form in the letter; expand the relevant case notes into full sentences. The letter should be 15-20 lines long. No more than the first 25 lines will be assessed.

Nursing Letter 10

Mr Lionel Ramamurthy, a 63-year-old, is a patient in the medical ward of which you are Charge Nurse.

Hospital: Newtown Public Hospital, 41 Main Street, Newtown

Patient details

Name: Lionel Ramamurthy (Mr)

Marital status: Widowed – spouse dec. 6 mths

Residence: Community Retirement Home, Newtown

Next of kin: Jake, engineer (37, married, 3 children <10)
Sean, teacher (30, married, working overseas, 1 infant)

Admission date: 04 February 2014

Discharge date: 11 February 2014

Diagnosis: Pneumonia

Past medical history: Osteoarthritis (mainly fingers) – Voltaren
Eyesight ↓ due to cataracts removed 16 mths ago – needs check-up

Social background: Retired school teacher (history, maths). Financially independent. Lonely since wife died. Weight loss – associated with poor diet.

Medical background: Admitted with pneumonia – acute shortness of breath (SOB), inspiratory and expiratory wheezing, persistent cough (→ chest & abdominal pain), fever, rigors, sleeplessness, generalised ache.
On admission – mobilising with pick-up frame, assist with ADLs (e.g., showering, dressing, etc.), very weak, ambulating only short distances with increasing shortness of breath on exertion (SOBOE).

Medical progress:	Afebrile. Inflammatory markers back to normal. Slow but independent walk & shower/toilet. Dry cough, some chest & abdom. pain. Weight gain post r/v by dietitian.
Nursing management:	Encourage oral fluids, proper nutrition. Ambulant as per physio r/v. Encourage chest physio (deep breathing & coughing exercises). Sitting preferred to lying down to ensure postural drainage.
Assessment:	Good progress overall
Discharge plan:	Paracetamol if necessary for chest/abdom. pain. Keep warm. Good nutrition – ↑ fluids, eggs, fruit, veg (needs help monitoring diet).

Writing Task:

Using the information given in the case notes, write a discharge letter to Ms Georgine Ponsford, Resident Community Nurse at the Community Retirement Home, 103 Light Street, Newtown. This letter will accompany Mr Ramamurthy back to the retirement home upon his discharge tomorrow.

In your answer:

- **Expand the relevant notes into complete sentences**
- **Do not use note form**
- **Use letter format**

The body of the letter should be approximately 180–200 words.

Nursing Letter 11

Patient Details

Patient: Maria Joseph is a 39 years old woman who has been a patient at a hospital you are working in as a head nurse. Apart from usual childhood illness such as chicken pox, she had been healthy.

10 / 5 2011

Subjective: Frontal headache for 6 hrs. Mild assoc, suffering from nausea, no vomiting, patient with blurred vision but not aura. No other symptoms noticed. She has no family history of migraine.

- **Objective** P96, BP 130/ 70. Normal Cervical Spine Movement, examination normal.
- **Assessment** Probably due to excess tension or personal dilemma
- **Plan** Advised to take rest. Given analgesia (paracetamol (500q4h))

14/5 /2011

Subjective Complained of continuous headaches (left sided and frontal), blurred vision, throbbing headache (left sided). Vomited 5 times during last three hours Complaining of slight paraesthesia.

- **Objective** Distressed, P 103, BP 150/90, Normal peripheral nervous system
- **Assessment** Severe Migraine Possibility
- **Plan:** Stat- Pethidine 100 mg, intramuscular injection Maxolon 10 mg

15 / 5 / 2011

Home Visit

Subjective Fell down at home due to severe left sided headache, started some 5 hrs after reaching home. Injured her right arm, bruises on left leg. slurred speech, half unconscious.

Objective P 100, BP 150/90, extension 4/5 power, left leg knee flexion 4/5

Assessment Probable intracranial pathology, space occupying lesions.

Plan Urgent assessment in Emer. Dept.

Using the information given above write a letter to the neurologist, who will attend the patient in the emergency department.

In your answer:

Expand the information given in complete sentences

Do not use note forms

Use only letter format.

The body of the letter should be approximately 180-200 words.

Read the case notes and complete the writing task which follows.

Notes

Hospital: Lyell McEwin Hospital

Patient Details: Name: Martin Wilson
Age: 62

Admission Date: 13 October 2009

Discharge Date: 24 October 2009

Diagnosis: Attempted suicide – overdose of Mogodol

Past Medical History: Heavy smoker (40 cigarettes/day)
Bronchitis (multiple episodes)
Underweight – 66kg, BMI 18
Psoriasis

Social History: Retired 2 years ago (bookkeeper with Holden Car Company)
Lives with wife, Joan, and adult son in housing trust maisonette in Elizabeth.
Wife works at Coles, son unemployed
2 married daughters and 5 grandchildren.
Regular social drinker
Depression related to gambling addiction
Began gambling 2 years ago
Has lost a lot of money including superannuation funds and is in debt.
Wife and family previously unaware of addiction – very angry but also upset about suicide attempt
Patient remorseful and ashamed
Wants to overcome addiction
Used to be a keen lawn bowls player
Has lost friends as result of gambling

Nursing Management: Weak and depressed. Anti-depressants prescribed – Lovan 200g
BP 130/95
Diagnosed with Type II diabetes.
Diabetes education regarding diet and oral medications
Wheelchair use from 20/10
Psoriasis on Torso and scalp – Diprosone OV cream 2x/day,
Ionil T Shampoo
Poor appetite
Physically unfit

Discharge Plan: Encouragement to maintain anti-depressant medication routine as the SSRI is established. Mrs Wilson will help with supervision
Monthly follow-up appointments with psychologist Dr Brian Murphy, Lyall McEwen Hospital
Social worker appointment to be made for gambling addiction therapy
Strong encouragement and assistance to join Gambling Addiction Action Group, Elizabeth Community Centre
Contact with Quitline needs to be encouraged
Wheel chair required for another week. Frame advised after this
Maintain psoriasis treatment
Maintenance of low GI diet for diabetes – involvement of wife necessary
Encouragement in social sporting activities eg lawn bowls?

Writing Task

Using the information in the notes, write a letter to the social worker, Ms Jennifer Adams, at the Elizabeth Community Health Centre, 125 Munno Parra Avenue, Elizabeth, 5098 requesting follow-up care. Stress that Mr Wilson's case needs urgent attention.

In your answer:

- expand the relevant case notes into complete sentences
- do not use note form
- use letter format

The body of the letter should be approximately 180-200 words.